



OFFICIAL

Pressure Injury Prevention and Management Procedure

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1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group	All services	Nursing, midwifery, medical, allied health

2. Purpose

The purpose of this document is to facilitate multidisciplinary quality care outcomes for prevention and management of pressure injury.

3. General information

A pressure injury is a localised injury to the skin and/or underlying tissue usually over a bony prominence, as a result of pressure, or pressure in combination with shear. A number of contributing or confounding factors are also associated with pressure injuries including microclimate, nutrition, tissue perfusion and oxygenation, moisture, co-morbidities, condition of the soft tissue, immobility and medical devices. The significance of these factors is yet to be elucidated.

The term pressure ulcer is synonymous with the term pressure injury (PI). Common sites for pressure injuries are identified in [Appendix 1](#) (lying) and [figure 4](#) (seated) positions.

Patients who are admitted will have a comprehensive assessment that encompasses their health status and cultural and environmental factors that may impact their wound healing or risk of developing wounds or pressure injuries while in hospital.

4. Risk assessment tools

Risk assessment is undertaken using the AKG Skin and Pressure Injury Assessment Tool AKMR121.9 which contains the Braden Scale and several other tools. The assessment tool is a guide to help identify individuals at risk of developing pressure injuries and must not replace clinical judgement. If they have an existing pressure injury, then by default they are 'at risk' e.g. if a patient has a pressure injury to the left heel, consider preventative strategies to protect the right heel.

4.1 Risk assessment

Risk assessment using AKMR 121.9 is to be performed within 8 hours of all patient admission to wards to identify those individuals at risk of developing a pressure injury.

4.2 Re-assessment occurs:

- With a change in the patient's condition or status
- Every 48 hours in the acute setting (as a minimum)
- Weekly in rehabilitation and long-term settings
- On all pre-operative patients and documented on the pre-operation check list

4.3 Braden scale for predicting pressure injury development

Clinical judgement is to be used in conjunction with the Braden Scale (AKMR 121.9) to determine management plan.

- If other major risk factors are present such as advanced age, fever, haemodynamic instability, impaired circulation, then advance to the next level of risk on Braden e.g. if Braden Score is 15 (at risk)
- For patients identified as being 'at risk' of developing pressure injury (due to Braden Score $\leq$ 18 or their clinical condition):
 - Inform Shift Coordinator, NUM, or Midwifery Manager (MM)
 - Implement the SSKIN Bundle and the interventions, initial actions and implement skin protection strategies of the Skin and Pressure Injury Assessment AKMR121.9
- Flag 'at risk' patients on iSOFT handover

NOTE: The lower the Braden Score, the greater the risk; however, any patient that is immobile, incontinent, inactive, has impaired cognition, impaired sensation or circulation or has a medical device should be considered 'at risk'. Additionally, if a patient has an existing pressure injury then by default they are 'at risk'.

5. SSKIN Bundle

A SSKIN bundle is a systematic tool to prompt staff to implement pressure injury prevention strategies to reduce the incidence of pressure injuries. It can also prevent a minor problem from becoming a major pressure injury. The SSKIN Bundle consists of:

Skin	• Skin inspection on each repositioning, including underneath medical devices and record each shift • Liaise with CNS Wound Care if advice required on ext. 1147 or mobile 0438936240
Surface	• Air mattress, cushions for chairs, heel elevators
Keep moving	• Encourage your patients to reposition themselves, reposition regularly, implement/document on <i>Pressure Ulcer Prevention Positioning Chart</i> AKMR 118A
Improve moisture management	• Meet patient's toileting or continence needs • Refer to CNS Wound Care if incontinence associated dermatitis present • Refer to Continence Services for continence associated services
Nutrition	• Weekly Malnutrition Screen Tool (MST) (<i>Skin and Pressure Injury Assessment</i> AKMR121.9) • Promote adequate hydration status and monitor patient's nutritional needs • Liaise with dietitian as required

5.1 Skin inspection

A comprehensive skin inspection is to be undertaken on:

- All patients within **8 hours** of admission using the *Skin and Pressure Injury Assessment* AKMR121.9
- Each shift (at a minimum) and on each repositioning. Document findings in the patient integrated notes, patient care plan, *Pressure Ulcer Prevention Positioning Chart* AKMR 118A
- If a wound or pressure injury is identified, stage the pressure injury as per Pressure Injury Classification System ([Appendix 4](#)) and commence a *wound management plan* AKMR 122. Refer to Wound Management Procedure
- Focus on skin overlying bony prominences as per Figure 1 and under medical devices
- Inspect skin for non-blanching erythema, re-position patient and reassess in 30 minutes. If persists this indicates a stage 1 PI
- Use opportunities such as showering/washing or turning the patient to inspect the skin

Assess for:

- skin temperature
- oedema
- change in tissue consistency in relation to surrounding tissue
- skin colour (areas of redness are a clinical sign of impending injury)
- can be more difficult to assess in darkly pigmented skin
 - Blanching erythema may be muted on dark pigmentated skin tones, instead staff can assess for areas that are darker than surrounding skin, shiny, or indurated.

Inspect skin each shift under and around medical devices:

- oxygen mask, nasal cannulae
- intravenous tubing
- nasogastric tubing
- indwelling catheter
- drains
- splints, braces, CAM boots and rigid collar
- anti-embolic stockings
- Ask patient to identify areas of discomfort or pain associated with pressure and assess these areas carefully.

Document findings in patient integrated notes and *Skin and Pressure Injury Assessment* AKMR 121.9 and in the patient care plan.

If a PI is identified, refer to the **interventions**, **initial actions** and **implement skin protection strategies** of the *Skin and Pressure Injury Assessment* AKMR121.9.

5.2 Surface

Place all patients at moderate to very high risk of developing a pressure injury on an alternating air mattress (unless contraindicated)

Document on patient care plan, *Skin and Pressure Injury Assessment* AKMR 121.9 and patient's integrated notes.

Air mattresses are contraindicated in:

- Spinal cord injured patients who have not received “radiological spinal clearance” from the consultant
- Patients with unstable fractures of the pelvis
- Patients requiring an ultra-low bed where the mattress overhangs the bed base creating an unstable base therefore are not suitable. Where a patient is a falls risk and at risk of pressure injury the risks need to be considered with the FRAMP assessment (AKMR 24) to determine if an ultra-low bed or an air mattress should be used
- Mattresses are appropriate for those who are immobile however their continued use must be reassessed as the patient’s status changes
- Hospital-owned pressure relieving mattresses may be available upon negotiation with the Rehabilitation and Sub Acute Medical Unit
- If no hospital air mattresses are available, refer to [Hiring and Cancelling of Air Mattresses Procedure](#)
- Ensure equipment is used and maintained according to manufacturer instructions

Note: An air mattress does not negate the need for regular patient repositioning or heel elevation

- Encourage patient to reposition themselves every 30 minutes; and staff to assist repositioning at a minimum of every 2-3 hours

5.3 Seating

- For patients with poor mobility, contact the Occupational Therapist (OT) for seating assessments. The OT will also assess and supply cushioning needs
- Avoid the use of cut outs/rings/donut devices/sheep skin and fluid filled gloves/bags as they localise pressure to other areas

5.4 Keep Moving

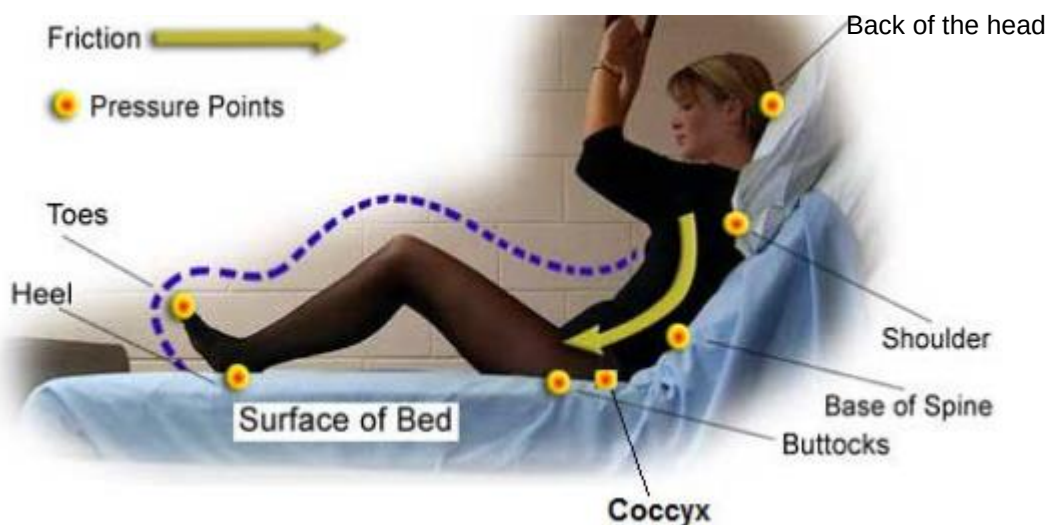
Repositioning

All patients who are at risk of developing a pressure injury or with an existing pressure injury should be encouraged to alter their position or be repositioned regularly unless contraindicated. The frequency for turning is based on the tolerance of skin for pressure, the individual’s preference, and clinical condition.

- Patients who are deemed high risk for developing a pressure injury must have pressure care performed a **minimum of 2-3 hourly**
- Caution is required with cognitively impaired patients whose levels of activity may fluctuate (e.g. hypo/hyperactive delirium)
- If unable to reposition patient due to therapy/clinical instability, document rationale for not repositioning in the patient’s integrated notes and inform shift coordinator / MO
- Encourage patients to reposition as appropriate and refer to Physiotherapist to teach patient to do ‘pressure relief lifts or other pressure relieving manoeuvres, every 30minutes
- Avoid positioning the patient on bony prominences
- Use manual handling aids to reduce friction and shear, such as slide sheets
- Position patients on a 30° lateral inclination alternating from side to side

- Avoid lying postures that increases pressure, such as the 90° side-lying position
- Avoid head-of-bed elevation or slouched position that places pressure and shear on the sacrum and coccyx
- Consider using the 'knee break' option on electric beds with this capability
- Consider postural alignment, distribution of weight, balance, stability, support of feet and pressure reduction when positioning individuals in seated positions (e.g. chairs, wheelchairs) see figure 4
- Document on patient care plan

Figure 4: Seated pressure points



Special considerations to prevent pressure injuries developing on heels:

- The posterior surface of the heel sustains intense pressure, even when a pressure redistribution surface (air mattress) is used. Assess patients for potential injury from bed end or attachments. Inspect the skin of the heels at each shift and document in patient care plan
- Ensure that the heels are free of the surface of the bed (i.e. use heel elevators). Refer to figure 5

Figure 5: Bed heel elevator



- Heel elevators (figure 5) are available on all wards – contact your Nurse Unit Manager
- Pillows or foam cushions used to elevate the heel should distribute weight of leg along the calf without placing undue tension on Achilles tendon. Knee should be in slight flexion unless contraindicated (e.g. following total knee replacement)
- Consider the use of a foam heel shaped dressing to minimise friction and shear e.g. Mepilex foam

5.5 Mobilisation

- Promote early mobilisation and increase activity as soon as tolerated
- Liaise with physiotherapist and OT for patients who are unable to mobilise
- Use patient mobilisation / ambulation as an opportunity to assess bony prominences that are not easily assessed in bed
- Restrict time sitting in chair if pressure injury risk factors are present

5.6 Improved Moisture Management

Skin Care

- Avoid positioning the patient on an area of erythema
- Keep the skin clean and dry
- Maintain a stable skin temperature
- Use a pH balanced skin cleanser
- Do not massage or rub skin vigorously
- Develop and implement an individualised continence management plan and document in nursing care plan
- Cleanse skin as soon as possible after incontinence
- Protect the skin from exposure to excessive moisture with a barrier product e.g. Sudocrem, Abena Skincare Ointment

Refer to [Bowel Management](#) Procedure and [Bladder Management](#) Procedure as appropriate.

- Use a skin moisturiser e.g. Abena Skincare Ointment to hydrate dry skin
- If medical device/prosthesis in situ – reposition (unless contraindicated) and perform skin assessment each shift. If unable to remove/reposition assess for pain/discomfort on/around pressure points (e.g. cervical collar, nasogastric tube, CAM boot)
- Document findings in patient integrated notes and nursing care plan

5.7 Nutrition

Nutrition screening:

- All admitted patients should be screened using the MST on admission, 48 hourly, and weekly in long term care settings (Rehabilitation, Banksia or Anderton) or with any change in patient's condition
- All patients considered high risk for malnutrition are to be placed on a pressure relieving mattress (malnutrition, immobility, reduced perfusion, and sensory loss are among the most important risk factors for developing pressure induced skin and soft tissue injuries)
- Refer patients to a dietitian:

- For full assessment of their nutritional status and requirements as per MST (MST ≥ 3) or if identified as at risk of developing a PI
- If identified with a PI stage 2 or deeper
- Consider need to monitor fluid and diet intake/output e.g. *Food Intake Chart* AKMR 60.2
- Liaise with MO as appropriate
- Measure and record weight (kg) weekly or as clinically indicated on *Adult Observation & Response Chart* AKMR140.3
- In consultation with dietitian maintain adequate nutrition through an individualised nutrition plan

6. Patient/carer education

- Educate patients and significant others about pressure injury risk development and prevention strategies
- Instruct patient/carer on how to inspect skin and to report concerns to healthcare provider. The patient/ carer should be considered an active participant in the management plan and should be informed of the relevant risk factors
- If care refused by patient – inform/educate patient (and carer if possible) of potential outcome, and document in the patient integrated notes

7. Management of pressure injury

- If a pressure injury is evident following skin inspection commence the patient on the SSKIN bundle
- Stage the pressure injury using the NPUAP/EPUAP Pressure Injury Staging System (Appendix 4)

Note: If a stage 1 pressure injury is identified, although a wound is not present there may be damage to the underlying tissue:

- Reassess and implement additional prevention strategies to avoid the pressure injury progressing to the next stage (e.g. heel elevators), Pressure Ulcer Prevention Positioning Chart AKMR 118A and increased frequency of repositioning
- A stage 1 pressure injury requires protection e.g. Mepilex foam or Mepilex Sacrum dressing
- Nursing staff to assess and order as required, the need for pressure relieving mattress/device unless contraindicated
- Refer to OT to assess and consider seating and supply pressure relieving equipment
- Pressure injuries on mucous membranes are unable to be staged and are often related to medical devices. Monitor and document on nursing care plan and integrated notes
- Develop/review Wound Management Plan AKMR122. Refer to Wound Management Procedure
- Reassess the pressure injury at each dressing change. If pressure injury Stage 3 or deeper present, or if the pressure injury shows no improvements or deteriorates, liaise with the multidisciplinary team and contact the CNS Wound Care, reassess the patient, pressure injury and document in the nursing care plan and integrated notes
- Assess the patient's level of pain and liaise with MO to ensure optimal management
- Review effectiveness of strategies each shift and adjust as appropriate

8. Documentation

Patient integrated notes to include:

- Skin assessment findings daily
- Pressure injury risk assessment
- Specific strategies employed to prevent or manage pressure injury (including those related to medical device management)

If pressure injury identified document:

- Anatomical location
- Stage (as per the NPUAP/EPUAP Pressure Injury Staging System (Appendix 4))
- Pressure support device or other equipment employed in management
- Wound Management Plan AKMR122
- Document all pressure injuries, (**Including Stage 1**) in the integrated notes. Clearly note if the pressure injury was present on admission. Complete an Electronic Clinical Incident report ([Datix CIMS](#)) and *CIMS Form Completed* sticker for **all** pressure injuries NOT present on admission.
- Document the presence of pressure injury, stage, anatomical location, and the plan for care in the medical record. Use the Pressure Injury Alert eForm on DMR (one per pressure injury).
- A new Pressure Injury Alert eForm and completion of [Datix CIMS](#) is required if the classification of a pressure injury changes (e.g. from stage II to stage III)
- A new Pressure Injury Alert eForm is needed each admission/discharge or transfer between organisations (but not on transfer from ward to ward)
- Update patient care plan as required
- Commence SSKIN Bundle as clinically indicated
- Record re-positioning regime on nursing care plan and Pressure Ulcer Prevention Positioning Chart AKMR 118A
- Update iSoft Clinical Manager (iCM)

Note: If a pressure injury occurs during care, the patient and /or carer should be informed in accordance with the [EMHS Open Disclosure Policy](#) principles and documented accordingly.

9. Handover

Patients who are identified of being at risk of developing a pressure injury and those with existing pressure injury to be communicated to staff during transition to new ward/shift change along with prevention and management plan.

- Update iSOFT

10. Discharge

Document the patient's most recent Braden risk assessment (adjust if clinical judgement determines actual risk is higher than Braden score) on the day of discharge or on transfer to another ward/facility.

- If patient has a pressure injury on discharge ensure a copy of *Wound Management Plan* AKMR 122 and Occupational Therapy Management Plan is communicated with the Residential Care Facility, general practitioner or home care providers

- Supply patient with Pressure Injury information for patient's pamphlet
- Ensure adequate dressing supply provided with patient until next dressing change

11. Policy context

The following documents are required to give effect to this policy:

- HDWA
 - [Clinical Handover Policy MP 0095](#)
- EMHS
 - [Open Disclosure Policy EMHS: 203](#)

12. Supporting information

The following documents support the implementation of this policy:

- [Hiring and Cancellation of Air Mattresses Procedure](#)
- [Bowel Management Guideline](#)
- [Bladder Management – Continence Procedure](#)
- [Occupational Therapy Pressure Injury Prevention and Management Procedure](#)
- [PCH Pressure Injury Prevention and Management Guideline](#)

13. Compliance, monitoring and evaluation

Compliance against this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS
- Auditing requirements via Combined Bedside and Risk Assessment Audit (CoBRA)

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunity for improvement.

Organisation-wide data trends and risk associated with pressure injury are to be reported to, and monitored by, the relevant governance committee.

14. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Comprehensive Care - Action 5.3, 5.21, 5.22, 5.23

15. References

1. Prevention and Treatment of Pressure Ulcers/Injuries: Clinical Practice Guideline. The International Guideline. European Pressure Ulcer Advisory Panel (EPUAP), National Pressure Injury Advisory Panel (NPIAP) and Pan Pacific Pressure Injury Alliance (PPPIA). 3rd Edition. 2019.
2. Cambridge Media Osborne Park, WA. Retrieved from https://www.woundsaustralia.com.au/Web/Resources/Publications/Publications_Users_Only/Pan_Pacific_Clinical_Practice_Guideline_for_the_Prevention_and_Management_of_Pressure_Injury_2012
3. Royal Perth Bentley Group – Clinical Practice Standard for Pressure Injury Prevention. Last Revision date: November 2020

16. Policy Owner

Director Nursing and Midwifery

Enquiries relating to this policy may be directed to:

Title: Clinical Nurse Specialist Stoma Wound Care
 Department: Surgical Services
 Email: ashleigh.shaw2@health.wa.gov.au

17. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

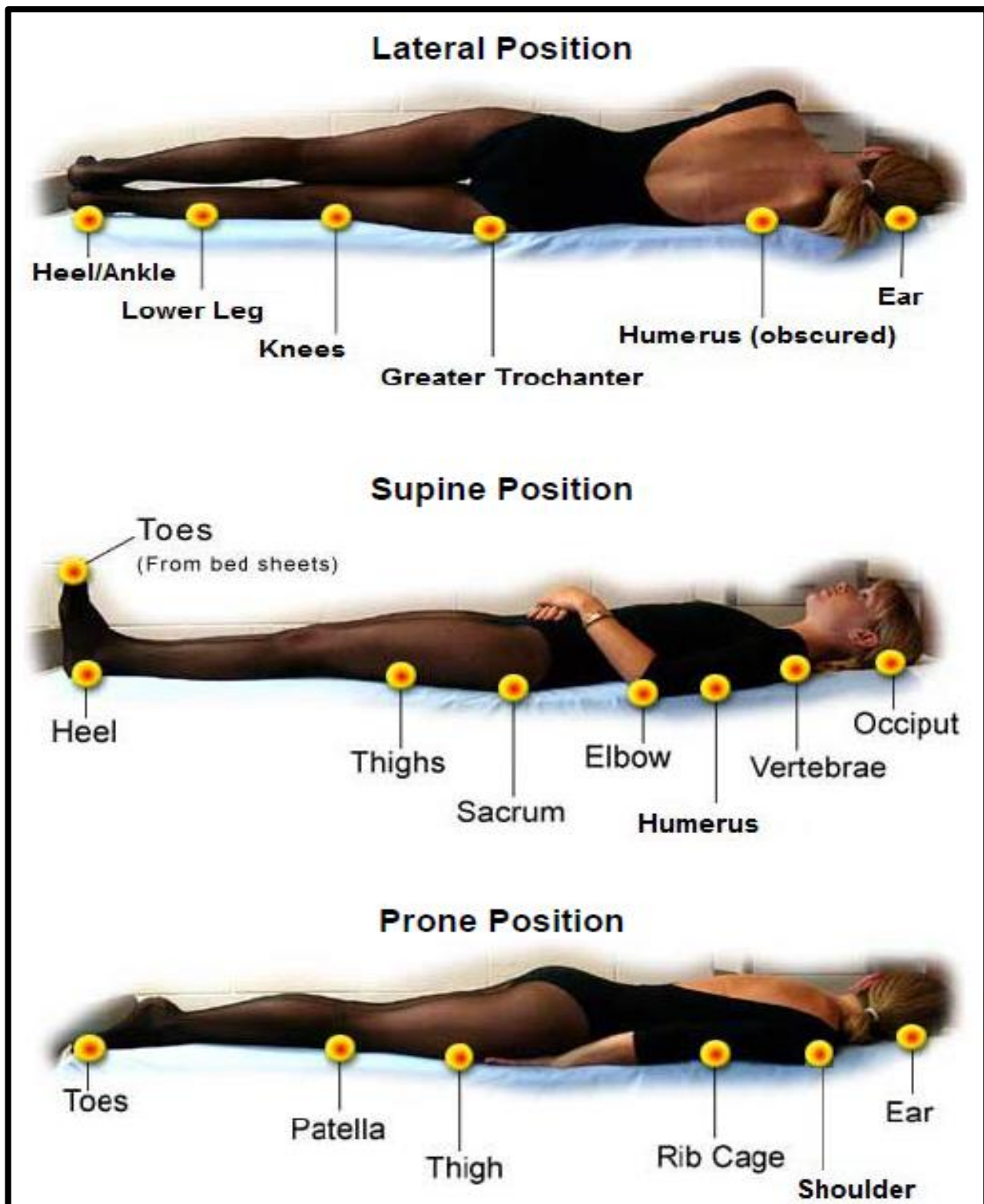
Version	Effective from	Effective to	Amendment(s)
V5	26 03 2026	26 03 2029	Full review, minor amendments only.

18. Authorisation

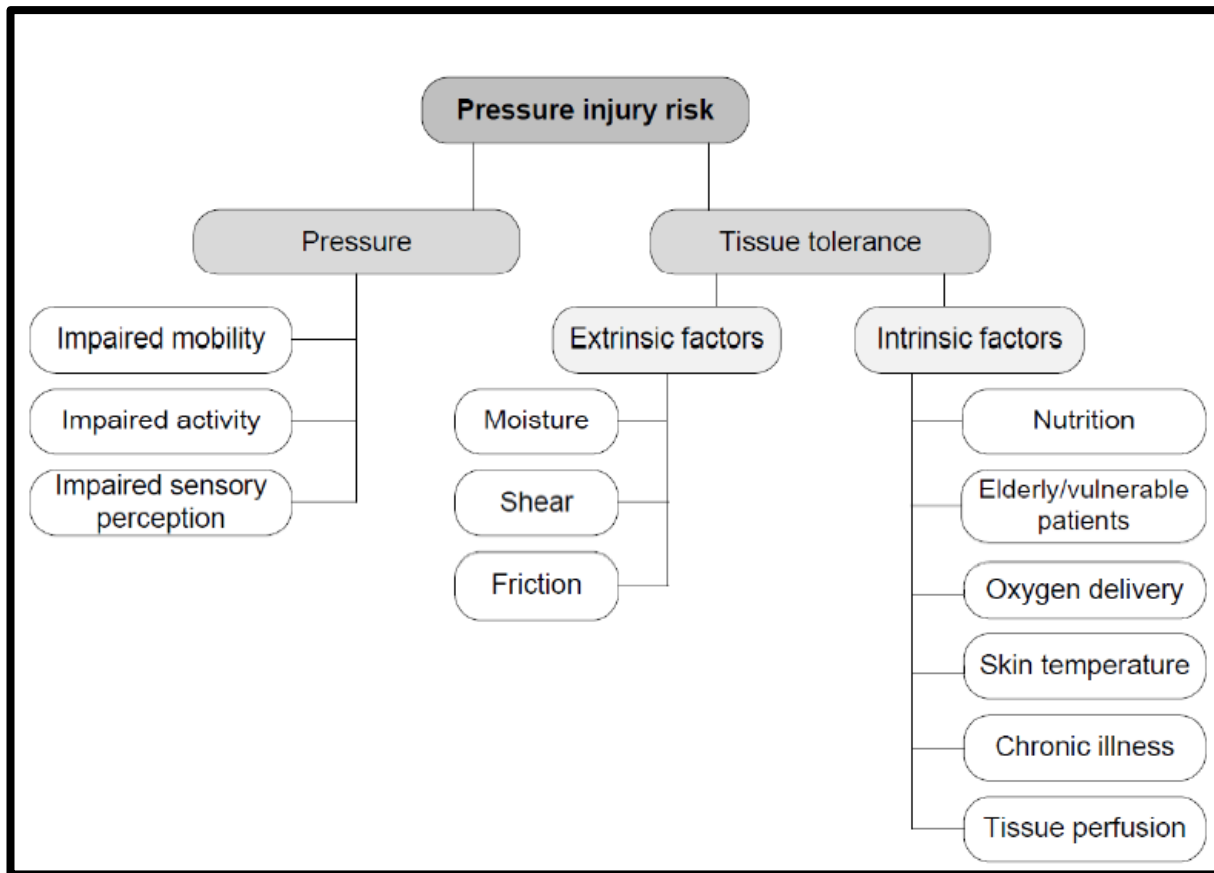
This policy has been authorised and issued by:

Authorised by	Russell Colyer-Cockburn – A/ Director Nursing & Midwifery
Authorisation date	25 03 2026
Published date	26 03 2026

Appendix 1: Common sites for PI while in lying position

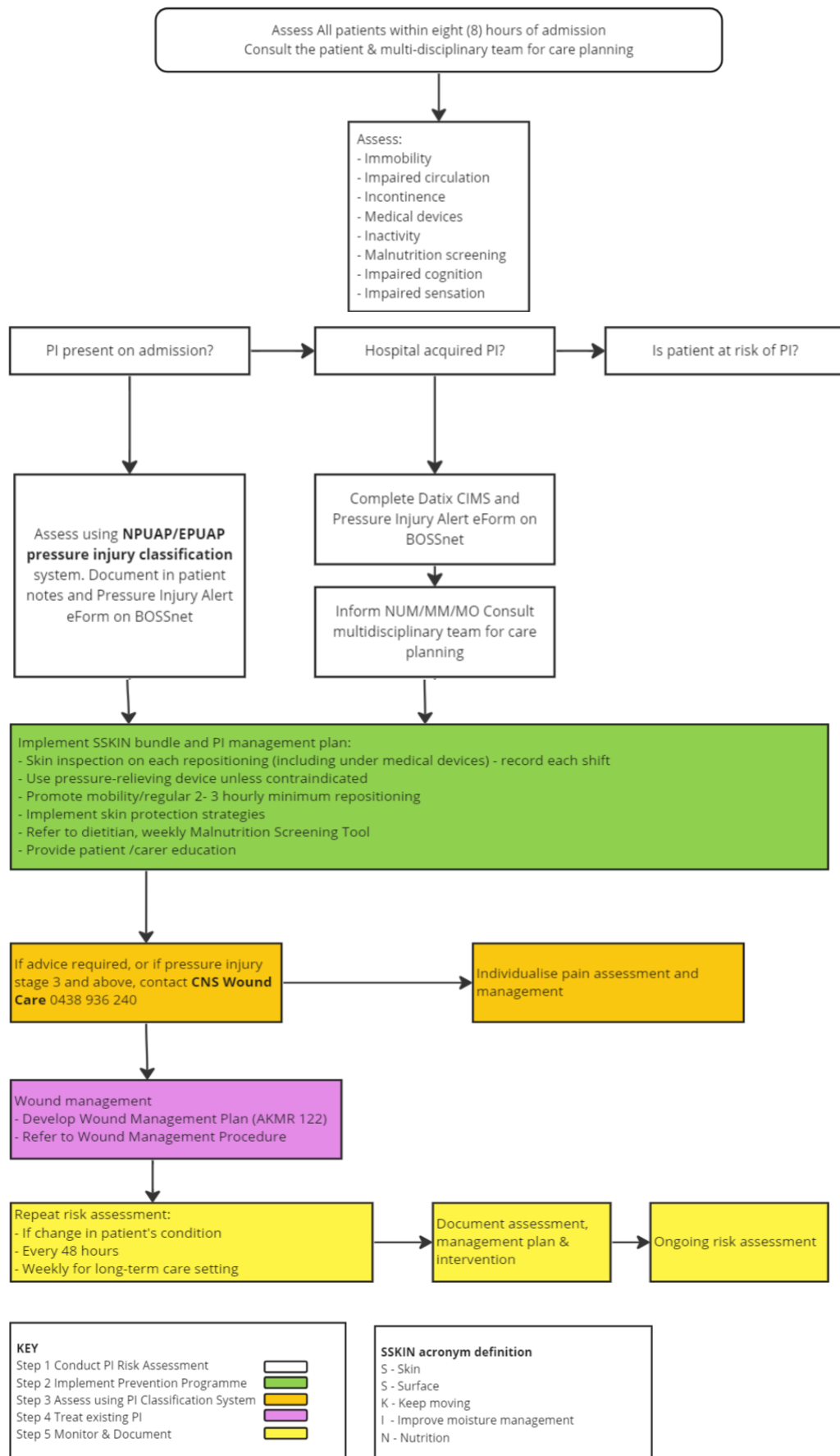


Appendix 2: Risk factors and risk assessment



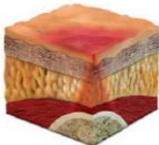
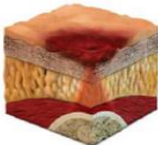
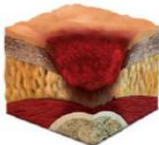
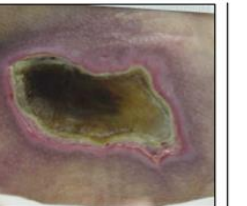
Source: Pan Pacific Practice Guideline for the Prevention and Management of Pressure Injury, 2012

Appendix 3: Risk factors and risk assessments Flow chart



Appendix 4: Pressure Injury Staging System

PRESSURE INJURY STAGING SYSTEM (NPUAP-EPUAP)¹.

Stage 1	Stage 2	Stage 3	Stage 4	Unstageable	Suspected Deep Tissue Injury
Intact skin with non-blanchable redness of a localised area usually over a bony prominence. Darkly pigmented skin may not have visible blanching; its colour may differ from the surrounding area. The area may be painful, firm, soft, warmer or cooler compared to adjacent tissue. May be difficult to detect in individuals with dark skin tones. May indicate "at risk" persons (a heralding sign of risk).	Partial thickness loss of dermis presenting as a shallow, open wound with a red-pink wound bed, without slough. May also be present as an intact or open/ruptured serum-filled blister. Presents as a shiny or dry, shallow injury without slough or bruising (NB bruising indicates suspected deep tissue injury). Stage 2 PI should not be used to describe skin tears, tape burns, perineal dermatitis, maceration or excoriation.	Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon or muscle are not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining or tunneling. The depth of Stage 3 PI varies by anatomical location. The bridge of the nose, ear, occiput & malleolus do not have subcutaneous tissue & Stage 3 PIs can be shallow. In contrast, areas of significant adiposity can develop extremely deep Stage 3 PIs. Bone or tendon is not visible or directly palpable.	Full thickness tissue loss with exposed bone, tendon or muscle. Slough or eschar may be present on some parts of the wound bed. The depth of Stage 4 PI varies by anatomical location. The bridge of the nose, ear, occiput & malleolus do not have subcutaneous tissue & Stage 4 PIs can be shallow. Stage 4 PI can extend into muscle and/or supporting structures (e.g. fascia, tendon or joint capsule) making osteomyelitis possible. Exposed bone or tendon is visible or directly palpable.	Depth Unknown Full thickness tissue loss in which the base of the PI is covered with slough (yellow, tan, grey, green or brown) and/or eschar (tan, brown or black) in the PI bed. Until enough slough/eschar is removed to expose the base of the PI, the true depth, & therefore the stage, cannot be determined. Stable (dry, adherent, intact without erythema or fluctuance) eschar on the heels serves as the body's natural biological cover & should not be removed.	Purple or maroon localised area or discoloured, intact skin or blood-filled blister due to damage of underlying soft tissue from pressure and/or shear. The area may be preceded by tissue that is painful, firm, mushy, boggy, warmer or cooler as compared to adjacent tissue. Deep tissue injury may be difficult to detect in individuals with dark skin tone. Evolution may include a thin blister over a dark wound bed. The PI may further involve & become covered by thin eschar. Evolution may be rapid, exposing additional layers of tissue even with optimal treatment.
					
					

1. National Pressure Ulcer Advisory Panel and European Pressure Ulcer Advisory Panel. 2009. Prevention and treatment of pressure ulcers: clinical practice guideline. National Pressure Ulcer Advisory Panel: Washington DC.
2. The Australian Wound Management Association (AWMA). 2012. Pan Pacific Clinical Practice Guideline for the Prevention and Management of Pressure Injury. Cambridge Publishing: Osborne Park, WA.
3. Diagrams reproduced with permission of AWMA © (2012). Wound images courtesy of WoundsWest.